

Joint Pain: Many Joints

(Polyarticular Joint Pain)

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Pain that involves more than just one joint is called polyarticular joint pain. A joint may simply be painful (arthralgia) or may also be inflamed (arthritis).

Pain that seems to be coming from joints can sometimes be coming from structures outside the joints, such as ligaments, tendons, or muscles (see [Introduction to the Biology of the Musculoskeletal System](#)). Examples of such disorders are [bursitis](#) and [tendinitis](#).

True joint pain (arthralgia) may or not be accompanied by joint inflammation (arthritis). The most common symptom of joint inflammation is pain. Inflamed joints may also be warm and swollen, and less often the overlying skin may be red. Arthritis may involve only joints of the limbs or also joints of the central part of the skeleton, such as the spine or pelvis. Pain may occur only when a joint is moved or may be present at rest. Other symptoms, such as rash, fever, eye pain, or mouth sores, may be present depending on the cause of the joint pain.

Different disorders tend to affect different numbers of joints. Because of this, doctors consider different causes of pain when the pain affects one joint (see [Joint Pain: Single Joint](#)) than when it affects more than one joint. When multiple joints are involved, some disorders are more likely to affect the same joint on both sides of the body (for example, both knees or both hands) than other disorders. This is termed symmetric arthritis. Also, in some disorders, an attack of arthritis remains in the same joints throughout the attack. In other disorders, the arthritis moves from joint to joint (migratory arthritis).

Causes of Pain in Many Joints

In most cases, the cause of pain originating inside multiple joints is arthritis. Disorders that cause arthritis may differ from each other in certain tendencies, such as the following:

- How many and which joints they usually involve
- Whether the central part of the skeleton, such as the spine or pelvis, is typically involved
- Whether arthritis is sudden (acute) or longstanding (chronic)

Acute arthritis affecting multiple joints is most often due to:

- Viral infection
- The beginning of a joint disorder or a flare up of an existing chronic joint disorder (such as [rheumatoid arthritis](#) or [psoriatic arthritis](#))
- [Gout](#) or [calcium pyrophosphate arthritis](#)

Less common causes of acute arthritis in multiple joints include [Lyme disease](#) and [gout](#) (disorders that also may affect only one joint), [gonorrhea](#) and [streptococcal bacterial infections](#), and [reactive arthritis](#) (arthritis that develops after an infection of the digestive or urinary tract).

Chronic arthritis affecting multiple joints is most often due to:

- Inflammatory disorders such as [rheumatoid arthritis](#), [psoriatic arthritis](#), or [systemic lupus erythematosus](#) (in adults)
- The noninflammatory disorder [osteoarthritis](#) (in adults)
- [Juvenile idiopathic arthritis](#) (in children)

Some chronic inflammatory disorders can affect the spine as well as the limb joints (called the peripheral joints). Some affect certain parts of the spine more frequently. For example, ankylosing spondylitis more commonly affects the lower (lumbar) part of the spine, whereas rheumatoid arthritis more typically affects the upper (cervical) part of the spine in the neck.

The most common disorders **outside the joints** that cause pain around the joints are:

- [Fibromyalgia](#)
- [Polymyalgia rheumatica](#)
- [Bursitis](#) or [tendinitis](#)

Bursitis and tendinitis often result from injury, usually affecting only one joint. However, certain disorders cause bursitis or tendinitis in many joints.

Evaluation of Pain in Many Joints

In evaluating joint pain, doctors first try to decide whether joint pain is caused by a disorder of the joints or a serious bodywide (systemic) illness. Serious bodywide disorders may need specific immediate treatment. The following information can help people decide when to see a doctor and know what to expect during the evaluation.

Warning signs

In people with pain in more than one joint, symptoms that should prompt rapid evaluation include:

- Joint swelling, warmth, and redness
- New skin rashes, spots, purple blotches, or nail pitting
- Sores in the mouth or nose or on the genitals
- Chest pain, shortness of breath, or new or severe cough
- Abdominal pain
- Fever, sweats, weight loss, or chills
- Eye redness or pain

When to see a doctor

People with warning signs should see a doctor right away. People without warning signs should call a doctor. The doctor decides how quickly they need to be seen based on the severity and location of pain, whether joints are swollen, whether the cause has been diagnosed previously, and other factors. Typically, a delay of several days or so is not harmful for people without warning signs.

What the doctor does

Doctors first ask questions about the person's symptoms and medical history. Then they do a physical examination. What doctors find during the history and physical examination often suggests a cause for joint pain and guides the tests that may need to be done (see table [Some Causes and Features of Pain in More Than One Joint](#))

Doctors ask about pain severity, onset (sudden or gradual), how symptoms vary over time, and what increases or decreases pain (for example, rest or movement or time of day when the symptoms worsen or abate). They ask about joint stiffness and swelling,

previously diagnosed joint disorders, and risk of exposure to sexually transmitted infections and Lyme disease.

Doctors then do a complete physical examination. They check all joints (including those of the spine) for swelling, redness, warmth, tenderness, and noises that are made when the joints are moved (called crepitus). The joints are moved through their full range of motion, first by the person without assistance (called active range of motion) and then by the doctor (called passive range of motion). This examination helps determine which structure is causing the pain and if inflammation is present. They also check the eyes, mouth, nose, and genital area for sores or other signs of inflammation. The skin is examined for rashes. Lymph nodes are felt and the lungs and heart examined. Doctors usually test function of the nervous system so that they can detect disorders of the muscles or nerves.

Some findings give helpful clues as to the cause. For example, if the tenderness is around the joint but not over the joint, bursitis or tendinitis is likely the cause. If tenderness is present in many areas besides the joints, fibromyalgia is possible. If the spine is tender as well as the joints, possible causes include osteoarthritis, reactive arthritis, ankylosing spondylitis, and psoriatic arthritis. Findings in the hand can help doctors differentiate between rheumatoid arthritis and osteoarthritis, two particularly common types of arthritis. For example, rheumatoid arthritis is more likely to involve the large knuckle joints (those that join the fingers with the hand) and wrist. Osteoarthritis is more likely to involve the finger joint near the fingernail. The wrist is unlikely to be affected in osteoarthritis, except at the base of the thumb.



Some Causes and Features of Pain in More Than One Joint

Cause	Common Features*	Tests†
Disorders usually causing symmetric joint pain		
Fibromyalgia	Joints not inflamed Chronic widespread pain and tenderness of muscles (that may involve joints and/or the back) Fatigue Sometimes irritable bowel syndrome or sleep disturbances Usually chronic, often affecting women Often depression or other mood disorders	Sometimes testing unnecessary
Infectious arthritis caused by viruses	Joint pain with or without inflammation, typically developing over hours or days Other symptoms of viral infection (for example, hepatitis B may cause jaundice, hepatitis C may cause purple blotches on legs, and HIV causes swollen lymph nodes)	Analysis of joint fluid Blood tests to identify the virus (most often hepatitis C or B or parvovirus)
Juvenile idiopathic arthritis	Chronic,‡ symmetric joint inflammation during childhood Lower back pain	Blood tests for autoantibodies§



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